FAX

Date:	02/27/2026
Pages including cover sheet:	8

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NOTE:

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Hello,
Please see attached wound care order for the following.
  *   Patient Name: Keith Gingrich
  *   Date of Birth: 06/24/1971
  *   Encounter Date: 02/26/2026, 08:30 AM CST
For any questions, please reply to this email and we will get back to
you shortly.
Best Regards,
Woundlocal Client Success Team
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Woundlocal - IndieMD TX Management, LLC
309 Water St Ste 107
Boerne, Texas - 78006-2866
Tel: 877-988-7420, Fax: 877-988-7420

Patient	: **Keith Gingrich**	Record Id :	**Indau2587**
Date of Birth	: **Jun 24, 1971**	Age / Gender :	**54 / M**
Provider	: **Kristy Garza, APRN-FNP-C**	Date of Visit :	**Feb 26, 2026**
Visit Type	: **Home Follow up/Conservative care**		

Chief Complaints

Follow up visit to re-evaluate lower back wound. Referral received from PAM health at home.

History of Present Illness

Treatment Note

Comprehensive Wound Care orders for nursing staff.

Wound #1: Mid lower back
- Cleanse wound with wound cleanser
- Pat dry only, do not scrub
- apply hydrogel then collagen
- Secure with bordered foam dressing. Pt unable to tolerate adhesives from other dressings.
- Frequency of dressing change: 3 times per week
- Expected duration of need. 2-4 weeks

Woundlocal next visit: 4 weeks (due to insurance coverage)
Home Health 2x/week for wound care and monitoring of wound
Wife to change the remaining day and as needed

Patient is located in: Home - 12
Hospice Patient: No
Is there a DME company involved in supplying supplies for this patient? No

54 Years male Keith Gingrich primary language is English and is being seen today for wound(s). Patient has condition(s) as listed below. Details about current wound(s) and any skin conditions are outlined below.
Pt with PMH of CAD, DM2, HTN, extensive orthopedic hx, s/p left knee surgery for fractured patella and torn tendon. Surgeon is Dr. Covington. Plan is for patient to return to surgery for knee replacement. Today left anterior knee with healed incision. No s/sx of infection. Pt reports wound to mid lower back that began 2 months ago. Pt's wife has been treating with collagen she received from orthopedic surgeon. She states wound appeared to be getting better but has not improved in the past 2-3 weeks. Today mid lower back wound with thick layer of slough to wound bed. Periwound without induration/erythema/fluctuance/warmth. Pt reports minimal pain. Recommend debridement of slough from wound bed to facilitate healing. Pt in agreement. Discussed POC. All questions answered.

10/21: Patient seen today in his home. He is A/Ox4, NAD, sitting up in recliner, he offers no complaints. Mid lower back wound with dressing intact, no strikethrough noted. Wound with less slough, increased

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granulation. Periwound without s/sx of infection. Pt reports he is drinking Juven 2x/day. Offloading and repositioning hourly while awake. Please see below for wound details.

10/30: Pt A/Ox4, NAD, up in recliner, offers no complaints. Pt had follow up visit with infectious disease MD. Antibiotics have been discontinued. The plan is for him to f/u with ortho surgeon for knee aspiration. If that shows no infection he will return to the OR for knee replacement. Dressing to mid lower back intact with no strikethrough. Wound bed with 100% slough. Periwound without erythema/induration/fluctuance/edema. No odor. No infection suspected. Pt reports good appetite. Discussed changes in treatment plan to address slough. All questions answered.

11/21: Pt seen in his home today. He presents sitting up in wheelchair, A/Ox4, NAD, reports intermittent pain to lower back wound. Dressing removed, no strikethrough, serosang draiange.Slough present to woundbed, not as thick, appears to be repainting to medihoney autolytic debridement. Periwound + erythema, warm, increased pain per patient. Wound culture obtained. Recommend starting oral antibiotic.

01/21: Pt seen for re-evaluation post hospitalization for left knee replacement surgery. He is now home, s/p knee replacement surgery complicated post operatively by infection. Pt reports 2 weeks ago incision opened and he was found to have infection and bleeding. He returned to OR and now presents with left knee wrapped and brace in place. Pt states dressing is to remain in place until his f/u with surgeon next week. There is no strikethrough to left knee dressing. Pt is ambulatory with a walker. Denies pain at this time. He confirms lower back wound is still present and open. Current treatment includes collagen and foam dressing. Dressing to lower back intact, no strikethrough. Periwound without evidence of infection. Minimal serosang exudate, no odor. Slough present to wound bed with areas of granular tissue. Do not suspect infection. Pt reports good appetite. Discussed POC. All questions answered.

02/26: No new issues or concerns related to lower back wound reported. Pt states wound seems to be getting smaller. Today mid lower back wound with increased epithelial to wound edges, + slough, + granulation, no odor. Do not suspect infection. Wound appears to be responding to current treatment. Recommend debridement to remove devitalized tissue and review wound healing. Pt in agreement. No changes in appetite reported. Continues supplemental protein. Pt to f/u in 2 weeks with ortho surgeon for post op left knee eval.

Active Medications:

- Cephalaxin (no end date) for prevention of infection

Active Allergies:

- No Known Drug Allergy
- No Known Food Allergy
- No Known Environmental Allergy
- No Known Latex Allergy

Pain associated with chief complaint: 2

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How is pain being addressed:
Topical analgesic.
denies pain

Has patient received:
Flu vaccine this year
- No, patient encouraged to get vaccinated.

Pneumococcal Vaccination ever
- No

Has patient fallen in the last 12 months: Yes
Past Medical History:
Coronary artery disease
Diabetes - type 2 - unspecified
Hyperlipidemia - unspecified
Hypertension
Muscle Weakness (Generalized)
Obesity
s/p left knee replacement with post op infection/bleeding

Medications taken that affect Wound healing
Pending documents from referral source.
NSAID
- Aspirin

ANTICOAGULANTS
- Aspirin

Cephalexin (no end date)

Family and Social History

Smoking status:
No
- Non smoker

Review of Systems

General: Denies weight change, fever, chills, general weakness, night sweats, fatigue
GI / Genitourinary Systems: No Active Issue
Appetite: Good
Is patient currently nutritionally compromised? No
Patient is Ambulatory w/Walker
Bed:
Pressure reduction mattress

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No Specialty Support Surface

Chair:
Pressure Reduction Cushion
No Specialty Support Surface

Feet:
In Bed
- Pillow

ROS reviewed: Yes
Lower extremity wound: No
Diabetic: Yes

Physical Examination

GENERAL: Alert and oriented x4, no acute distress, follows directions, well nourished.
SKIN: Intact except for open wound noted below. Nailbeds pink, no cyanosis.
HEENT: Normocephalic, PERRLA. Conjunctivae are clear.
RESP: Even and unlabored.
PSYCH: Appropriate mood and affect.

Extremities:
Generalized weakness noted.
Left knee unable to bend due to issues with kneecap and ligaments. Pt to return for reconstructive surgery of left knee joint in December.

Wound #1 Location: Mid lower back
Primary Etiology:
Pressure
- Stage 3 - Full thickness ulcer extending through dermis into the subcutaneous tissue.

Tissue Level of Wound:
Epidermis
Dermis
SubQ
Slough

Tissue Types present %:

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Slough
- 20
Granulation
- 80

Wound Description:
Edges:
- Flat
Amount of drainage:
- None to minimal
Exudate:
- Sero-Sanguinous
Periwound:
- Intact

Evidence of Osteomyelitis: No
Date Wound Began (mm/dd/yy): 10/14/2025
Initial Measurement in cm: 2.0x1.7x0.2 cm
Date Conservative Care began (mm/dd/yy): 10/14/2025
Post CC measurements in cm: 2.0x1.5x0.2 cm (11/21/2025)
Last Week's measurement in cm: 1.5x1.5x0.1 cm (01/21/2026)
Today's Measurements in cm: 1.1x1.2x0.1 cm (02/26/2026)
Did wound fail to respond to >4 weeks of conservative wound care? Yes, Collagen, debridement, calcium alginate, medihoney, offloading
Wound size greater than 1cm sq? Yes
Evidence of Infection: No
Treatment Objective: Healing/Maintain Healing
Wound Status: Stable / At goal
Goals of care discussed, and decision made to continue conservative care.
DEBRIDEMENT PROCEDURE:
Debridement Consent: Risks and complications (including bleeding/pain/infection) were discussed with the patient. - The patient understands that there is no guarantee that the wound will heal. - Consent was obtained from
- Patient.
Wound was cleansed with
- Wound Cleanser
Anesthesia was obtained with
- 20% benzocaine spray
Debridement to remove devitalized tissue was performed using a
- 4mm curette
Debridement was to the level of
- subcutaneous tissue.
Percent of nonviable tissue remaining:
- 0-10 percent

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Bleeding was minimal. Hemostasis was obtained. Clean dressing was applied.
Patient tolerated procedure well.
Post-operative recommendations and updates to the plan of care are documented in the assessment and plan.
Post debridement depth:
1.1x1.2x0.2 cm

Is wound eligible for grafting:
No
due to insurance coverage

Is patient compliant with off-loading? Yes, Offloads lower back with pillows, repositions Q1-2 hrs

Assessment Notes

1. Wound #1 Mid lower back stage 3 pressure injury- Measuring smaller, less slough, increased granulation since last evaluation. Continue hydrogel and collagen. Cont foam dressing as this is he only type of dressing that does not cause skin reaction. No infection suspected.
2. Offloading measures: Recommend repositioning and offloading Q2H
3. ABI n/a
4. Supplements Recommendations: Juven 2x/day
5. Diabetes management: Managed by PCP- Dr. Teofilo Sanchez.
6. Next Woundlocal visit f/u in 4 weeks.

Diagnoses

Diagnosis	Code	Comments
Pressure ulcer of unspecified part of back, stage 3	L89.103	

Diets

Recommend that patient increase protein intake and supplements listed above.

Kristy Garza, FNP-C

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Electronically Signed By Kristy Garza, APRN-FNP-C [NPI: 1659848364] on Feb 27, 2026 08:19 AM